

From: Richard D Smith em@editorialmanager.com
Subject: Your Submission SSM-D-26-04899
Date: 21. July 2026 at 6:16 PM
To: Hector Bahamonde hector.bahamonde@utu.fi

RS

Ref: SSM-D-26-04899

Manuscript Title: When Delegation Meets Framing: Experimental Evidence on Mass Preferences over High-Cost Cancer Medicines
Social Science & Medicine

Dear Dr. Bahamonde,

Thank you for submitting your manuscript to Social Science & Medicine. We have now completed its review and attach the comments of the reviewers. Your paper is of potential interest, however based on the reviewers' comments, we cannot accept your manuscript in its present form. As you will see our reviewers raised a number of suggestions for significant revision and clarification.

If you feel able to respond to these suggestions, we would be willing to consider a revised submission. Please submit your revision through the on-line system by Aug 20, 2026.

If we have not heard from you by Aug 20, 2026, we will assume you do not wish to submit a revision and your manuscript will be withdrawn. If you feel you will need more time to revise and re-submit your paper, please let us know immediately. Extensions to resubmission deadlines are only granted in exceptional circumstances.

To submit a revision, go to <https://www2.cloud.editorialmanager.com/ssm/default2.aspx> and click 'log in' at the top of the page. There you will see a menu item called "Submissions Needing Revision". You will find your submission record there. When submitting your revision you will be asked to upload a response to the reviewers' comments. Please also include a document that highlights the tracked changes made to your original submission.

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All revisions should adhere to the same format as initial submissions. In addition to addressing the reviewers comments, please remember to:

1. Double space the manuscript.
2. Remove any identifying information from the main text manuscript and supporting documents (i.e. response to reviewers). (Acknowledgements should be submitted as a separate file, which will be omitted from the review PDF).
3. Please use the APA system for formatting references.
4. Please remember that the abstract, tables, and references as well as the main text of all resubmissions should be within our word limit.
5. Please ensure there are no endnotes or footnotes.
6. When you upload your revision and response to reviewers, please put your response to reviewers first and your revised manuscript and tracked changes document second.
7. Do not upload previous versions of your manuscript, as we already have copies of these.

We look forward to hearing from you.

Kind regards

Kind regards,

Senior Editor
Richard D Smith
Social Science & Medicine

Additional comments from the Editors and Reviewers (if any):

Reviewer #1: The manuscript addresses an important question about public preferences for funding high cost cancer medicines. The paper is clearly written and the literature review is strong. The topic fits the journal. However, the central claim is larger than the evidence can support.

1. The main concern is that the central claim is not aligned with the experimental design. The manuscript argues that institutional insulation does not neutralize framing effects, but the experiment only varies the description of the funding decision. It does not vary who makes the decision or how the decision is institutionally justified. Respondents are not assigned to conditions in which the decision is made by politicians, an expert agency, an HTA body, or another public authority. They are also not told whether the decision is part of an insulated expert process. For this reason, the study can show that different descriptions affect stated funding preferences, but it cannot show whether institutional insulation moderates or fails to moderate framing effects.

2. The Highlights appear to belong to another version of the manuscript. They mention variables such as ideology and institutional trust, which are not measured in the study. This should be corrected before the manuscript can be evaluated further.

3. The treatment arms do not only use different frames. They also give respondents different information. The rescue arm adds information about limited treatment options. The health maximisation arm adds information about opportunity costs. Therefore, the estimated differences may reflect information effects, not only framing effects.

4. The two treatment arms may operate through different mechanisms. The rescue condition may increase support because it highlights patients with few remaining options. The health maximisation condition may reduce support because it highlights opportunity costs and cuts to other healthcare services. These mechanisms are not equivalent to a simple gain versus loss contrast. The manuscript should therefore be more cautious in attributing the full treatment difference to prospect theory.

5. The control condition is shorter than the two treatment conditions. This creates a possible text length and task difficulty problem. Both treatment arms increase Don't know responses. This pattern may reflect added information, increased complexity, or respondent uncertainty.

6. The authors should report full sample models that include Don't know responses. They should also report sensitivity analyses that show whether the main conclusions depend on excluding these respondents. The increase in Don't know responses should be discussed as a substantive result. It may show uncertainty, confusion, ambivalence, or lower confidence in the decision. It should not be treated only as a missing or unusable response.

7. The ordinal logit model assumes that each predictor has the same effect across the response thresholds. This assumption may not hold here because the frames may affect rejection, conditional funding, and unconditional funding in different ways. The authors should test the proportional odds assumption or use a model that relaxes it. Reporting a multinomial logit model is useful, but it does not by itself show that the ordinal logit assumption is valid.

8. The manuscript should show the raw outcome distributions by experimental arm. Readers need to see the basic differences across groups before interpreting model based estimates.

severe interpreting model based estimates.

9. The regression reporting should be expanded. The full model table should include covariates, thresholds, sample sizes, fit statistics, weighting details, and confidence intervals. Figure 2 should be supported by a table with exact predicted probabilities and 95 percent confidence intervals.

10. The use of survey weights needs a clearer explanation. The sample comes from a non probability online panel with quota recruitment. Weights can improve balance with population margins, but they do not provide standard design based population inference. The authors should report unweighted results and soften claims about representativeness.

11. The contribution to the existing literature needs to be clearer. Prior studies have already examined framing, opportunity costs, and public preferences in priority setting. Since this study does not test institutional insulation, the authors should explain its contribution as a focused framing study.

12. The response scale may channel different views into the conditional funding category. Some respondents may choose it because they support price negotiation. Others may choose it because they are uncertain, because they want to avoid denying treatment, or because they view unconditional funding as too permissive. These are substantively different reasons, but the current measure cannot distinguish them. The manuscript should therefore discuss the modal conditional funding result as a possible measurement issue and should be cautious in drawing policy implications from it.

13. Several parts of the Discussion go beyond the evidence. The manuscript discusses political trust, ideology, system satisfaction, industry lobbying, patient organisations, and patient influencers. These topics are relevant to the broader policy area, but most are not measured in this study. The Discussion should be shortened and tied more closely to the results.

14. The vignette wording needs clarification. The phrase about laboratory evidence destroying cancer cells may lead respondents to infer stronger clinical benefit than intended. The authors should clarify the source and meaning of this wording.

15. The manuscript does not include manipulation checks or mechanism measures. The authors interpret the treatment effects as evidence of loss avoidance, gain preservation, and the rule of rescue, but the survey does not measure whether respondents actually perceived the treatments in these terms. It does not measure perceived patient loss, perceived opportunity cost, perceived absence of alternatives, or perceived moral obligation. The evidence therefore supports a treatment effect on stated funding preferences, but it does not identify the psychological mechanism behind that effect. The mechanism claims should be softened unless additional evidence is provided.

Overall, the manuscript has potential as a focused survey experiment on framing and public preferences for high cost cancer medicine funding. The key tasks are to correct the Highlights, narrow the central claim, address the framing and information confound, reanalyse Don't know responses, and improve statistical reporting.

Reviewer #2: This appears to me to be a well constructed piece of research, which will contribute to the ongoing discussion between what you refer to as the 'rescue' vs 'health maximisation' approach. One major point I feel you have misjudged is that cancer treatment today is often, and in some types of cancers, almost always about extending life rather than 'curing' the disease.

A very minor point: you use 'should' on a couple of occasions, I believe a less normative word of words would be better e.g. 'would' or 'probably'

Data in Brief (optional):

We invite you to convert your supplementary data (or a part of it) into an additional journal publication in Data in Brief, a multi-disciplinary open access journal. Data in Brief articles are a fantastic way to describe supplementary data and associated metadata, or full raw datasets deposited in an external repository, which are otherwise unnoticed. A Data in Brief article (which will be reviewed, formatted, indexed, and given a DOI) will make your data easier to find, reproduce, and cite.

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Response to Reviewers (mandatory)

This should be a separate file labeled "Response to Reviewers" that carefully addresses, point-by-point, the issues raised in the comments appended below. You should also include a suitable rebuttal to any specific request for change that you have not made. Mention the page, paragraph, and line number of any revisions that are made. Please do not include any author identifying information in this file as it is viewed by reviewers.

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